PoH | Quintubate

A video-laryngoscope costs lakhs. A doctor's idea brings it to thousands.



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DR. MAANAS JAIN
FOUNDER AND CTO, QUINTUBATE

PEOPLE OF HEALTHCARE

People of Healthcare is connecting with leaders and builders across healthcare and MedTech. This is one of them.



Dr. Maanas Jain

Founder and CTO, Quintubate Pvt. Ltd.

MBBS, AIIMS Jodhpur
Internal Medicine, Rutgers University, USA
Before starting residency, he set out to build Quintubate.

Sources: Dr. Jain’s Instagram profile (@quintubate); his answers to People of Healthcare.

A laryngoscope costs ₹1,000 to 2,000. A video-laryngoscope costs lakhs.

DRAWN TO SCALE (₹)

Standard laryngoscope

₹1,000 to 2,000

UVEAL attachment (company figure)

starts under ₹10,000

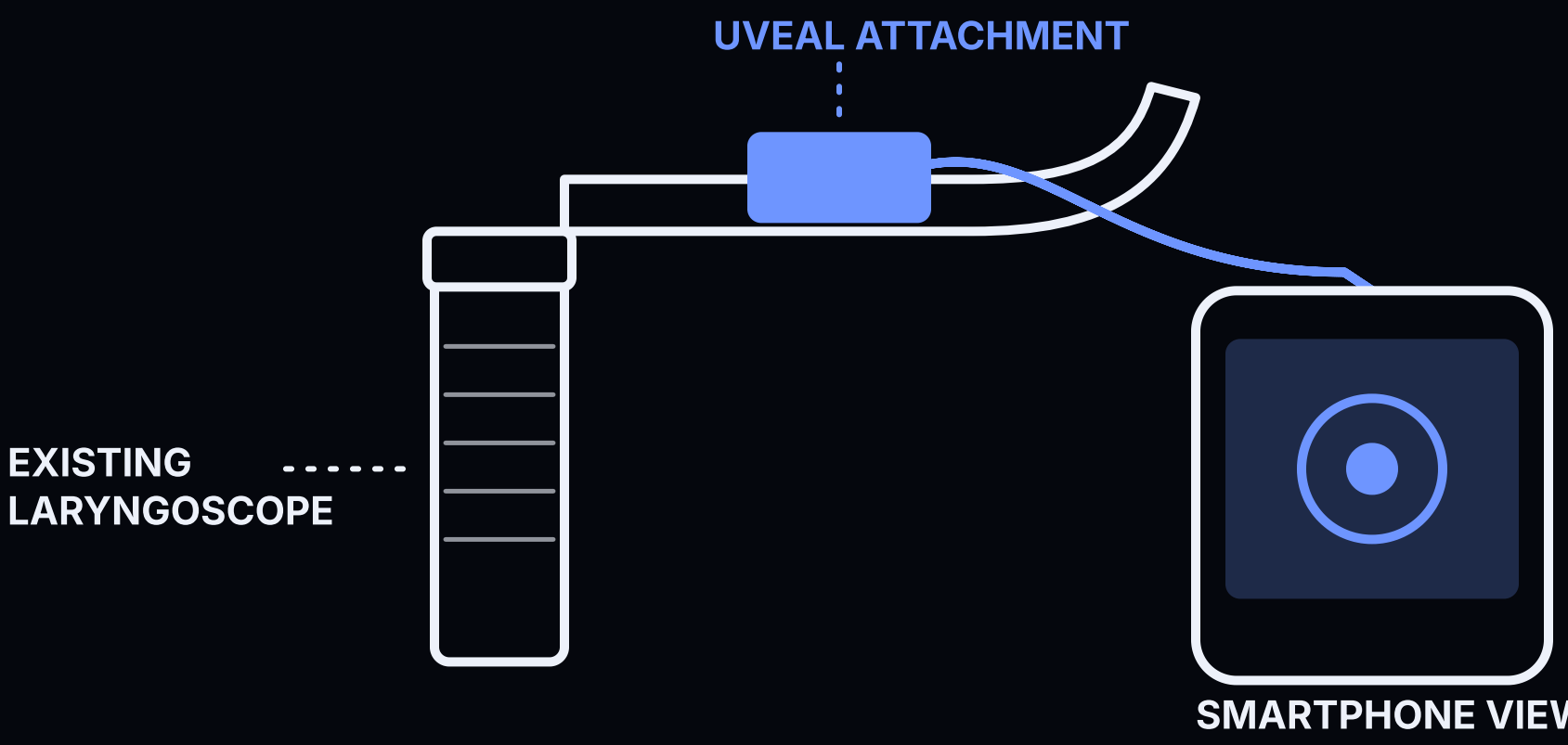
Video-laryngoscope (as shown in his video)



₹2,00,000 to ₹3,00,000

Drawn to scale. Video-laryngoscope figures as shown in Dr. Jain's own video; the UVEAL figure is the company's.

UVEAL turns the laryngoscope a hospital already has into a video-laryngoscope.



How it works: the laryngoscope already in the hospital, the attachment, and the phone already in the doctor’s pocket. Illustration, not to scale.

A real intubation, at a government hospital.



UVEAL Channeled Scopes

A Real Intubation with UVEAL at Civil hospital, Gandhinagar

A real intubation with UVEAL at Civil Hospital, Gandhinagar (Dr. Jain’s video). The company states the device is CDSCO certified. Dr. Jain says 250+ doctors across India and 12 other countries use UVEAL.

What are you building, and what do you want the world to know?

“Quintubate is an initiative focussed on democratizing access to high-tech, expensive medical devices... at a sustainable cost for Indian healthcare.”

“UVEAL brings down the cost of videolaryngoscopy from the lakhs to the thousands. In a span of 6 months since commercialization, UVEAL is being used by 250+ doctors across India and 12 other countries.”

What are the bottlenecks you face?

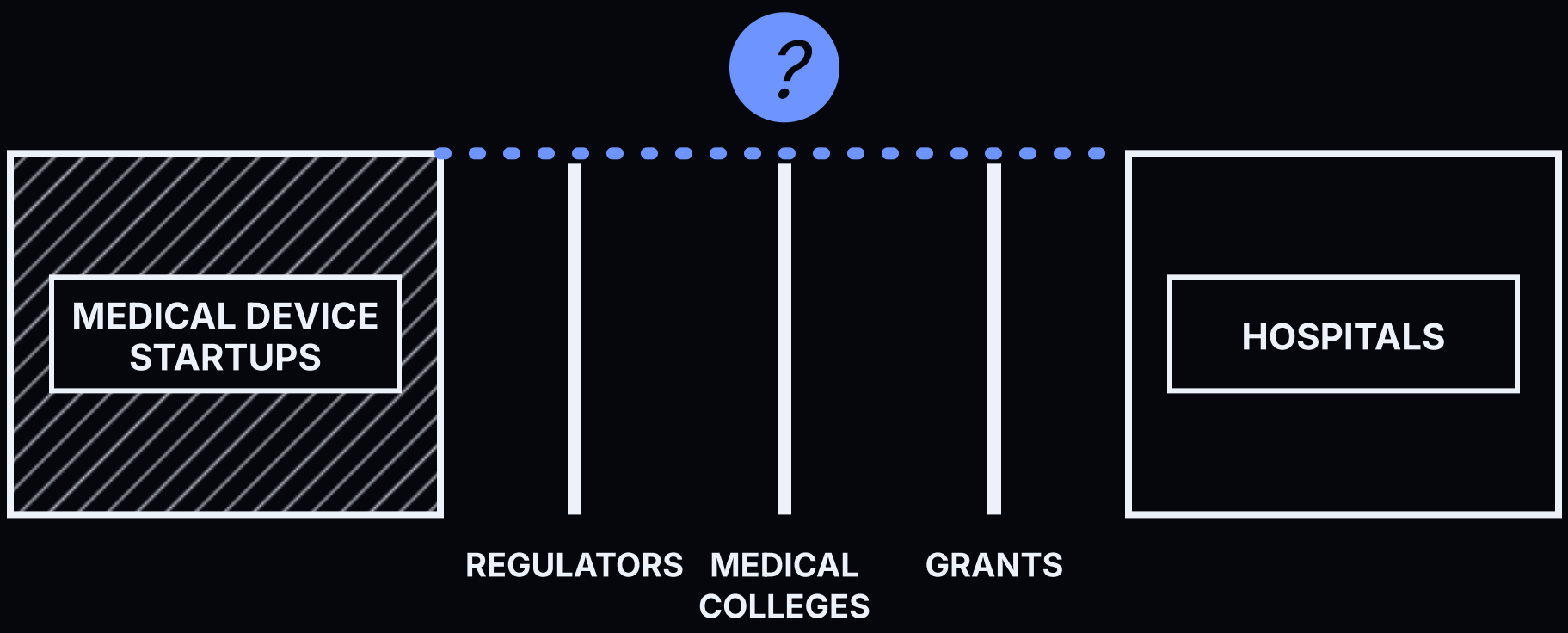
“The medical device ecosystem is largely limited to a select few companies, all based internationally, and hence healthcare in India is not supportive of made-in-India devices. Clinicians are not aware, and are skeptical of Indian medical device startups.”

What bridges need to be built, and how do we increase adoption?

“There needs to be an ecosystem for hospitals to learn about new licensed products developed by medical device startups as well as an incentive to adopt such indigenously designed devices. Regulators and medical colleges need to be on the lookout and supportive.”

“Fostering dialogue between medical device startups and existing hospitals would greatly enhance the chances of adoption.”

Who builds the bridge between the startup and the hospital?



An illustration of the gap Dr. Jain describes. Regulators, medical colleges and grants are the supports he names.

How do you see the future of healthcare, and what needs to be done?

“If Indian [doctors and hospitals] do not push for development and adoption of indigenous devices, medical device innovation in India will inevitably stagnate.”

“There is a big opportunity. Since very few medical devices are actually made in India, there is a huge scope for founders to pitch in and innovate to fill that large gap.”

What he needs right now.

Meetings with hospitals and administrators, to explain the device and how it can be procured.

Access to grants for new prototype development.

Introductions to hospital administrators and government healthcare officials.

A doctor sees a problem at the bedside. What happens next depends on whether anyone makes room for the solution.

Building it is one part. The next step needs hospitals willing to look, grants, and a culture that adopts what is built here.



The conversation with Dr. Jain.

Some doctors don’t wait. They build.

Know a hospital administrator, regulator or grant-maker who should meet Dr. Jain?

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